# Pain Management Procedure

## 1. Scope

| Site | Operational Area | Applicable to |
|---|---|---|
| Armadale Kalamunda Group | All services | Nursing, Medical and Allied Health |

## 2. Purpose

The purpose of this document is to establish a generic overview of pain medicine interventions which enable activities of daily living to be performed with the minimum amount of discomfort at Armadale Kalamunda Group (AKG).

This document outlines:

- Pharmacological and non-pharmacological techniques
- Multi modal techniques and multidisciplinary approaches e.g. allied health professions such as Physiotherapy, Clinical Psychology and Occupational therapy.

For more detailed information refer to: ANZCA Acute Pain Management Guidelines

## 3. Background

### 3.1 Inclusions: pain assessment

#### 3.1.1 Pharmacological interventions

- Oral preparations
- Intramuscular Medication
- Intravenous medication
- Patient controlled analgesia – intravenous, epidural or regional
- Epidural Analgesia
- Regional analgesia
- Entonox
- Transdermal opioid patches

#### 3.1.2 Non-Pharmacological interventions

- Transcutaneous Electrical Nerve Stimulation
- Heat/ Cold Therapy

#### 3.1.3 Allied Health

- Clinical Psychology
- Physiotherapy
- Occupational therapy
- Outpatient CBT programs

### 3.2 Exclusions

- Implanted Intrathecal therapy
- Addiction Medicine
- Palliative Care

All staff are required to be appropriately credentialed prior to the administration of the following medication related to this policy.

- Epidural
- Intravenous administration of opioids via bolus dosing and/or continuous infusion
- Programming of Patient controlled intravenous analgesia devices
- Entonox administration

## 4. Procedure

### 4.1 Pain assessment

Continuous assessment and documenting of patient's pain intensity scores are vital to identify action required for appropriate and effective management. It is imperative that effective and frequently evaluated pain protocols are put in place.

The correct assessment and management of patient's pain in conjunction with the evaluation of the effectiveness of interventions given is fundamental to pain management. Pain should be assessed both on movement and at rest and a multi modal medication regime which assists with "normal" function prescribed

The baseline assessment of pain should include:

- General medical and surgical history, background of persistent pain and recent or past use of recreational drugs
- Physical examination including heart rate, blood pressure, respiratory rate. Range of movement
- Patient self-reporting of pain
- Location of pain
- Intensity of pain (using a recognised, appropriate pain scale), rest, Movement and procedural pain
- Consistency of assessment is important, and the same tool should be used. This score to be documented on appropriate observation chart. The evaluation of analgesic effectiveness can also be determined using this method and should be repeated 30-40 mins post analgesia.

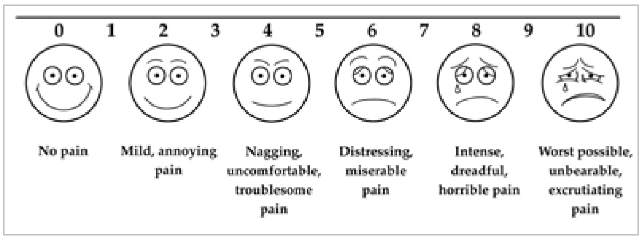
## 4.2 Escalation for review

Patients who self-report an unexplained increase in pain or have no reduction in pain score post administration of multimodal treatments should be reviewed by the Pain management team or treating medical officer. Recognition of signs and symptoms of adverse effects of treatment must be reported as per AKG policy and National Safety and Quality Health Service Standard 9.

## 4.3 Pain assessment tools

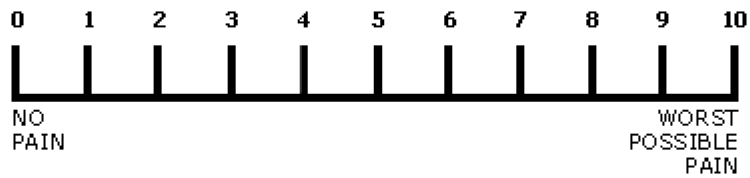
### 4.3.1 Visual analogue scale (VAS)

The far left face indicates ‘no pain’ and the right ‘worst pain ever’.



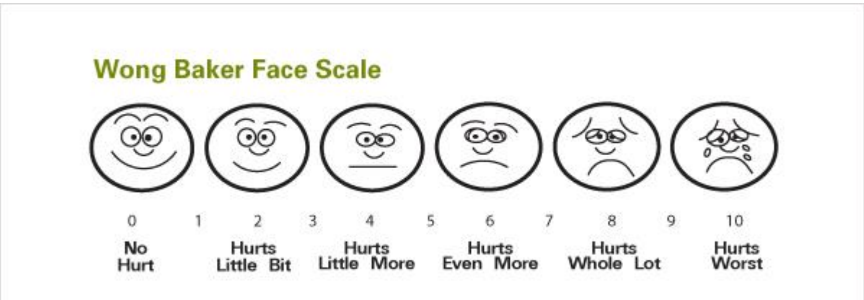

### 4.3.2 Numerical rating scale (NRS)

Where 0 is no pain and 10 is worst possible




### 4.3.3 Face rating scales (FRC)

This is useful for patients who find the numbered charts difficult, it addresses “how you feel”.




### 4.3.4 Behavioural rating scale (BRS)

Designed for use in non-verbal patients

| | Score = 0 | Score = 1 | Score = 2 | Cumulative Score |
|---|---|---|---|---|
| **Face** | Face muscles relaxed | Facial Muscle tension, frown, grimace | Frequent to constant frown, clenching of jaw | Face Score: |
| **Restlessness** | Quiet, relaxed appearance, normal movement | Occasional restless movement, shifting position | Frequent restless movement may include extremities or head | Restlessness Score: |
| **Muscle tone** | Normal muscle tone | Increased tone, flexion of fingers and toes | Rigid tone | Muscle tone score: |
| **Vocalisation** | No abnormal sounds | Occasional moans, cries, whimpers and grunts | Frequent or continuous moans, cries, whimpers or grunts | Vocalisation score: |
| **Consolability** | Content and relaxed | Reassured by touching, distractible | Difficult to comfort by touch or talk | Consolability Score: |
| **Total Score** | | | | **/10** |

### 4.3.5 Functional activity score (FAS)

Ask patient to perform a task related to their painful area, observe them during this activity and score A, B, C.

- A= No limitation
- B= Mild limitation
- C= Severe limitation

### 4.3.6 Pain Assessment for Advanced Dementia patients (PAINAD)

- Refer to Appendix 1 Tool

## 5. Pharmacological interventions

Following formal assessment of the patient's pain score, prescribed medication can be administered as per Medication policy

- Prescribed medications may be administered by appropriately credentialed staff. These include registered nurses, medication-endorsed enrolled nurses, physiotherapists, medical practitioners.
- All staff members must follow the Six rights of Medication Administration

| Technique | Uses | Observations/Cautions |
|---|---|---|
| Oral Administration | Administration of analgesic agents is simple; noninvasive, has good efficacy in most | • Refer to WHO analgesic ladder<br>• Reduced gastric emptying |

| Technique | Uses | Observations/Cautions |
| --- | --- | --- |
| | settings and high patient acceptability | • Opioid-induced paralytic ileus |
| Intramuscular Administration | The intramuscular route of opioid administration should be discouraged and is not the recommended initial treatment for moderate to severe pain | • Pain score, BP, Pulse, respiration rate, SP02 and sedation score or conscious state (AVPU)<br>• Temperature 4 hourly<br>• 5 min during bolus administration<br>• 0-2 hours every 30 mins<br>• 2-4 Hours every 1 hour<br>• 4-8 hours every 2 hours<br>• + 8 Hours every 4 hours or as clinically indicated[5] |
| Intravenous Medications | The benefit of intravenous opioid medication is the rapid onset of action compared to other forms of delivery | • Pain score, BP, Pulse, respiration rate, SA02 and Sedation score or Conscious state (AVPU)<br>• Limited availability. Refer to specific protocols for administration in critical care areas only. |
| Epidural Analgesia | Epidural analgesia provides pain relief by administration of local anaesthetics and /or opioids into the epidural space via a catheter | • Pain score, BP, Pulse, respiration rate,SA02 and Sedation score or Conscious state (AVPU)<br>• Temperature 4 hourly<br>• Sensory/Motor blockade 8 hourly, directly following top up or as clinically indicated<br>• Refer to Labour Ward epidural policy<br>• Signs of local anaesthetic toxicity, which may include:<br>1. Light headedness<br>2. Dizziness<br>3. Visual and auditory disturbances (difficulty focusing and tinnitus)<br>4. Disorientation<br>5. Drowsiness<br>6. Muscle twitching<br>7. Seizure<br>8. Coma<br>9. Respiratory depression or arrest<br>10. Hypotension<br>• 0-2 hours every 30 mins |

| Technique | Uses | Observations/Cautions |
| --- | --- | --- |
| | | • 2-4 Hours every 1 hour<br>• 4-8 hours every 2 hours<br>• + 8 Hours every 4 hours or as clinically indicated[5]<br>• Visual check of insertion site 8 hourly or as clinically indicated |
| Regional Analgesia | Regional analgesia is the blocking of pain pathways from a circumscribed area of the body by placing local anaesthetic solution via a catheter next to the nerves that supply the area | • Pain score, BP, Pulse, respiration rate, SA02 and Sedation score or Conscious score (AVPU)<br>• Temperature 4 hourly<br>• Signs of local anaesthetic toxicity (see epidural cautions)<br>• 0-2 hours every 30 mins<br>• 2-4 Hours every 1 hour<br>• 4-8 hours every 2 hours<br>• + 8 Hours every 4 hours or as clinically indicated[5]<br>• Visual check of insertion site 8-hourly or as clinically indicated |
| Entonox | Entonox is a colourless, odourless gas comprising of a mixture of nitrous oxide and oxygen. It is patient controlled and administered via inhalation.<br>Longer term use (outside of labour) may require prophylactic administration of B12 and folic acid | • Observe for signs of:<br>• Effectiveness of analgesia<br>• Level of responsiveness<br>• Analgesia<br>• Light anaesthesia |
| Transdermal Opioid Patches | Transdermal opioid patches can be used to deliver a specific dose of opioid medication over a number of days, e.g. Buprenorphine patches can remain in situ for 7 days. | • Rotation of sites<br>• Sign and date on application<br>• Ensure smooth and complete adherence to skin<br>• Regular pain scoring<br>• Site should be checked each shift and documented to show awareness of medication when administering other analgesia |
| Epidural Steroid Injection | This is used as a treatment for radiculopathy and is placed near the affected nerve root. | • Regular pain scores |

| Technique | Uses | Observations/Cautions |
|---|---|---|
| Nerve Block | Induction of anaesthesia in a specific part of the body by injecting a local anaesthetic close to the sensory nerves that supply it | |
| Trigger Point Injections | Trigger points are tight bands or small areas of a muscle that are sensitive to touch and pressure. The trigger point can irritate surrounding nerves and cause referred pain, a type of pain that radiates to nearby areas | |
| Facet Joint Injection | Are non-surgical treatments to relieve pain and inflammation of the facet joints in the spine | |
| Peripheral Joint Injection | Are administered within the joints of the shoulder, elbow, hip and knee. Pain in these joints may be caused by bursitis, tendonitis or arthritis. | |

### 5.1 Non pharmacological

| Technique | When used/excluded |
|---|---|
| Heat Therapy | The application of heat can improve physical function, reduce pain intensity and improve pain relief when used in conjunction with other therapies |
| Cold Therapy | The principle effect of cooling living tissue is a reduction in metabolic rate which lowers activity in all tissues by reducing oxygen uptake |
| Transcutaneous Electrical Nerve Stimulation (TENS) | TENS is a non- pharmacological technique that can be used alongside other treatments for the person who has persistent/acute pain offering more choice of available proven therapies on offer to assist in the management of pain |
| Physiotherapy | Assessment of patient mobility and provide education regarding walking aid etc. Assessment of pain and appropriate mobilisation to assist in pain management. |

## 6. Persistent pain

### 6.1 Allied health

A multidisciplinary approach to the management of pain, can lead to improved pain relief, patient outcomes and reduced incidence of side effects, such an approach is recommended for all patients, especially those with complex medical or psychological pathology.

#### 6.1.1 Physiotherapy

- Physiotherapists play a critical role in assisting people to live with persistent pain.
- Physiotherapists work across the healthcare settings with the aim of diminishing pain, improving quality of life where possible and preventing acute and sub-acute painful conditions developing into persistent pain.
- Physiotherapists work directly with the other disciplines within the Pain Medicine Unit, namely Pain Specialist Doctors, Clinical Psychologists and Occupational Therapists in applying a biopsychosocial approach and facilitate the knowledge and skills necessary for patients to self-manage pain.
- The Physiotherapist's role related to patient care within the multi-disciplinary team is to assess the level of functional ability and impairments of the patient, determine the appropriate physiotherapy management (including referral to short or long pain management courses) and communicate overall management strategies with other members of the Pain Team.
- The Physiotherapist will have individual consultations and work in delivering and developing the short and long group programmes.

#### 6.1.2 Role

- Perform a thorough assessment including listening to patients' story.
- Teach and reinforce stretching, strengthening and fitness exercise.
- Correct and retrain poor movement strategies. Only utilise manual therapy short term and only if really necessary
- Teach and apply pacing principles to improve activity levels
- Educate and explain basics of pain physiology relevant to patient
- Refer to interdisciplinary members and /or short or long pain management courses as appropriate

### 6.2 Occupational Therapy

#### 6.2.1 Role:

- Develop pacing Strategies.
- Individualizing SMART goals for pacing and short-term goal setting
- General management of persistent pain
- Access to Occupational Therapy options / supports.
- Increasing the functional capacity of people with pain
- Focus on SMART goals for valued activity.
- Specific individual training in activity management, goal setting, and problem-solving, including return to the workforce (volunteer or paid), where appropriate

## 7. Patient education

Patient education is fundamental to the effective management of pain, not only the physiological but the psychological aspects need to be understood by the patient and their family. Access to educational workshops and additional services should be provided.

## 8. Policy context

The following documents are required to give effect to this procedure:

- HDWA
  - Consent to Treatment Policy
- EMHS
  - Patient Identification and Procedure Matching EMHS: 21
  - Patient Consent Policy

## 9. Supporting information

The following documents support the implementation of this procedure:

- Communicating for Safety Procedure
- ANZCA Acute Pain Management Guidelines

## 10. Compliance, monitoring and evaluation

Compliance against this procedure will monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.
- Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.
- Organisation-wide data trends and risks associated with pain management are to be reported to, and monitored by, the Comprehensive Care Steering Committee.

## 11. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 4 – Action 4.3 Partnering with consumers.
- Standard 6 – Action 6.8 Clinical handover
- Standard 8 – Action 8.4 Recognising acute deterioration.
- Standard 8 – Action 8.6 Escalating care

## 12. References

1. ANZCA PS41 Guideline on acute pain management PS41(G)-Acute-pain-2023.pdf

## 13. Glossary

The following definitions are relevant to this policy.

| Term | Definition |
|---|---|
| Pain | The International Association for the Study of Pain (IASP) defines pain as "an unpleasant sensory and emotional experience associated with actual or potential tissue damage or described in terms of such damage"; it is subjective and can be influenced by previous pain experiences, beliefs about pain, culture, mood and ability to cope |

## 14. Policy owner (relating to these procedures)

**Director Clinical Services**

Enquiries relating to this procedure may be directed to:

Title: Head of Anaesthesia
Department: Anaesthesia
Contact: Vanessa.Percival@health.wa.gov.au

## 15. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

| Version | Effective from | Effective to | Amendment(s) |
|---|---|---|---|
| V3.1 | 03 02 2026 | 14 03 2028 | Minor update to add PAINAD as an appendix. |

## 16. Authorisation

This procedure has been authorised and issued by:

| | |
|---|---|
| **Authorised by** | Dr Tania Strickland – Director Clinical Services |
| **Authorisation date** | 03 02 2026 |
| **Published date** | 03 02 2026 |

# Appendix 1

## Pain Assessment in Advanced Dementia Scale (PAINAD)

***Instructions:*** Observe the patient for five minutes before scoring his or her behaviors. Score the behaviors according to the following chart. Definitions of each item are provided on the following page. The patient can be observed under different conditions (e.g., at rest, during a pleasant activity, during caregiving, after the administration of pain medication).

| Behavior | 0 | 1 | 2 | Score |
|---|---|---|---|---|
| Breathing<br>Independent of vocalization | • Normal | • Occasional labored breathing<br>• Short period of hyperventilation | • Noisy labored breathing<br>• Long period of hyperventilation<br>• Cheyne-Stokes respirations | |
| Negative vocalization | • None | • Occasional moan or groan<br>• Low-level speech with a negative or disapproving quality | • Repeated troubled calling out<br>• Loud moaning or groaning<br>• Crying | |
| Facial expression | • Smiling or inexpressive | • Sad<br>• Frightened<br>• Frown | • Facial grimacing | |
| Body language | • Relaxed | • Tense<br>• Distressed pacing<br>• Fidgeting | • Rigid<br>• Fists clenched<br>• Knees pulled up<br>• Pulling or pushing away<br>• Striking out | |
| Consolability | • No need to console | • Distracted or reassured by voice or touch | • Unable to console, distract, or reassure | |
| | | | **TOTAL SCORE** | |

(Warden et al., 2003)

***Scoring:***
The total score ranges from 0-10 points. A possible interpretation of the scores is: 1-3=mild pain; 4-6=moderate pain; 7-10=severe pain. These ranges are based on a standard 0-10 scale of pain, but have not been substantiated in the literature for this tool.

***Source:***
Warden V, Hurley AC, Volicer L. Development and psychometric evaluation of the Pain Assessment in Advanced Dementia (PAINAD) scale. *J Am Med Dir Assoc*. 2003;4(1):9-15.

## PAINAD Item Definitions

(Warden et al., 2003)

**Breathing**

1. *Normal breathing* is characterized by effortless, quiet, rhythmic (smooth) respirations.
2. *Occasional labored breathing* is characterized by episodic bursts of harsh, difficult, or wearing respirations.
3. *Short period of hyperventilation* is characterized by intervals of rapid, deep breaths lasting a short period of time.
4. *Noisy labored breathing* is characterized by negative-sounding respirations on inspiration or expiration. They may be loud, gurgling, wheezing. They appear strenuous or wearing.
5. *Long period of hyperventilation* is characterized by an excessive rate and depth of respirations lasting a considerable time.
6. *Cheyne-Stokes respirations* are characterized by rhythmic waxing and waning of breathing from very deep to shallow respirations with periods of apnea (cessation of breathing).

**Negative Vocalization**

1. *None* is characterized by speech or vocalization that has a neutral or pleasant quality.
2. *Occasional moan or groan* is characterized by mournful or murmuring sounds, wails, or laments. Groaning is characterized by louder than usual inarticulate involuntary sounds, often abruptly beginning and ending.
3. *Low level speech with a negative or disapproving quality* is characterized by muttering, mumbling, whining, grumbling, or swearing in a low volume with a complaining, sarcastic, or caustic tone.
4. *Repeated troubled calling out* is characterized by phrases or words being used over and over in a tone that suggests anxiety, uneasiness, or distress.
5. *Loud moaning or groaning* is characterized by mournful or murmuring sounds, wails, or laments in much louder than usual volume. Loud groaning is characterized by louder than usual inarticulate involuntary sounds, often abruptly beginning and ending.
6. *Crying* is characterized by an utterance of emotion accompanied by tears. There may be sobbing or quiet weeping.

**Facial Expression**

1. *Smiling or inexpressive*. Smiling is characterized by upturned corners of the mouth, brightening of the eyes, and a look of pleasure or contentment. Inexpressive refers to a neutral, at ease, relaxed, or blank look.
2. *Sad* is characterized by an unhappy, lonesome, sorrowful, or dejected look. There may be tears in the eyes.
3. *Frightened* is characterized by a look of fear, alarm, or heightened anxiety. Eyes appear wide open.
4. *Frown* is characterized by a downward turn of the corners of the mouth. Increased facial wrinkling in the forehead and around the mouth may appear.
5. *Facial grimacing* is characterized by a distorted, distressed look. The brow is more wrinkled, as is the area around the mouth. Eyes may be squeezed shut.

**Body Language**

1. *Relaxed* is characterized by a calm, restful, mellow appearance. The person seems to be taking it easy.
2. *Tense* is characterized by a strained, apprehensive, or worried appearance. The jaw may be clenched. (Exclude any contractures.)
3. *Distressed pacing* is characterized by activity that seems unsettled. There may be a fearful, worried, or disturbed element present. The rate may be faster or slower.
4. *Fidgeting* is characterized by restless movement. Squirming about or wiggling in the chair may occur. The person might be hitching a chair across the room. Repetitive touching, tugging, or rubbing body parts can also be observed.
5. *Rigid* is characterized by stiffening of the body. The arms and/or legs are tight and inflexible. The trunk may appear straight and unyielding. (Exclude any contractures.)
6. *Fists clenched* is characterized by tightly closed hands. They may be opened and closed repeatedly or held tightly shut.
7. *Knees pulled up* is characterized by flexing the legs and drawing the knees up toward the chest. An overall troubled appearance. (Exclude any contractures.)
8. *Pulling or pushing away* is characterized by resistiveness upon approach or to care. The person is trying to escape by yanking or wrenching him- or herself free or shoving you away.
9. *Striking out* is characterized by hitting, kicking, grabbing, punching, biting, or other form of personal assault.

**Consolability**

1. *No need to console* is characterized by a sense of well-being. The person appears content.
2. *Distracted or reassured by voice or touch* is characterized by a disruption in the behavior when the person is spoken to or touched. The behavior stops during the period of interaction, with no indication that the person is at all distressed.
3. *Unable to console, distract, or reassure* is characterized by the inability to soothe the person or stop a behavior with words or actions. No amount of comforting, verbal or physical, will alleviate the behavior.